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7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name : **ZUBAIR SHAIKH**
Ref By : **DR. VIKRAM JAIN**
Reg No : **HL6201830845 / MH602**
Barcode : **HEN123793**
INV : Allergic Bronchopulmonary Aspergillosis (ABPA) Panel

Age : 50 Year | **Sex :** Male
Registered on : 30-08-2026 08:47 AM
Received on : 30-08-2026 04:50 PM
Reported on : 31-08-2026 01:01 PM

SAMPLE COLLECTED AT :
SNS PATHO LAB

SAMPLE : Serum

ASPERGILLUS FUMIGATUS SPECIFIC IGE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
ASPERGILLUS fumigatus IgE	0.08	kU/L	. - .

Reference Range

Class	IgE kU/L	Interpretation
0	<0.35	Negative
1	0.35-0.69	Equivocal
2	0.70-3.49	Positive
3	3.50-17.4	Positive
4	17.5-49.9	Strongly positive
5	50.0-99.9	Strongly positive
6	> or =100	Strongly positive

Clinical significance :

Clinical manifestations of immediate hypersensitivity (allergic) diseases are caused by the release of proinflammatory mediators (histamine, leukotrienes, and prostaglandins) from immunoglobulin E (IgE)-sensitized effector cells (mast cells and basophils) when cell-bound IgE antibodies interact with allergen.

In vitro serum testing for IgE antibodies provides an indication of the immune response to allergens that may be associated with allergic disease.

The allergens chosen for testing often depend upon the age of the patient, history of allergen exposure, season of the year, and clinical manifestations. In individuals predisposed to develop allergic disease, the sequence of sensitization and clinical manifestations proceed as follows: eczema and respiratory disease (rhinitis and bronchospasm) in infants and children less than 5 years due to food sensitivity (milk, egg, soy, and wheat proteins) followed by respiratory disease (rhinitis and asthma) in older children and adults due to sensitivity to inhalant allergens (dust mite, mold, and pollen inhalants).

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **VINOD HYALIJ**	**Age :** 65 Y | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. BETTERFEEL LAB**	**Registered on** : 31-08-2026 12:15 AM	
Reg No	: **HL4301831630 / MH004/MS121**	**Received on** : 31-08-2026 08:27 AM	
Barcode	: **HEJ089010**	**Reported on** : 03-09-2026 02:54 PM	
INV	: CD3-CD4-CD8-CD45		**SAMPLE :** EDTA Blood

CD3-CD4-CD8-CD45

TEST DESCRIPTION	RESULT	UNITS	RANGES
Absolute CD3 + (T-Lymphocytes) Method: Flowcytometry	874.71	Cell/uL	1007-2479
% CD3+ / CD45 (T–cells) Method: Flowcytometry	78.44	%	59.43-84.36
Absolute CD4 + (T-Helper cells) Method: Flowcytometry	284.69	Cell/uL	371-1217
% CD3+/CD4 + (T-Helper cells) Method: Flowcytometry	25.53	%	27.90-50.50
Absolute CD8 + (T-Suppressor cells) Method: Flowcytometry	552.3	Cell/uL	355-1171
% CD3+/CD8 + (T-Suppressor cells) Method: Flowcytometry	49.52	%	18.47-40.83
CD4/CD8 ratio	0.52	.	0.72-2.10

Interpretation:

When the CD4 count falls below 400 cells/mcL, HIV-positive patients can be diagnosed with AIDS and can receive antiretroviral therapy.
When the CD4 count falls below 200 cells/mcL, prophylaxis against Pneumocystis Jiroveci pneumonia is recommended.

Clinical Information:

Lymphocyte subset counts should be appropriately interpreted in context of the clinical presentation and other immunological parameters and relevant laboratory test results.
Lymphocytes in peripheral blood (circulation) are heterogeneous and can be broadly classified into T cells, B cells, and natural killer cells. There are various subsets of each of these individual populations with specific cell-surface markers and function. This assay provides absolute (cells/mcL) and relative (%) quantitation for total T cells and CD4+ and CD8+
Abnormalities in the number and percent of CD3, CD4, and CD8 T cells have been described in a number of different disease conditions. In patients who are infected with HIV, the CD4 count is measured for AIDS diagnosis and for initiation for antiviral therapy. The progressive loss of CD4 T-lymphocytes in patients infected with HIV is associated with increased infections and complications.
The absolute counts of lymphocyte subsets are known to be influenced by a variety of biological factors, including hormones, the environment, and temperature.

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REPORT

PT Name : **ANVI PARAB**
Ref By : **DR. CELINE D'MONTE (FIGER)**
Reg No : **HL6101832629 / HYMH/NALSPR/MILL1**
Barcode : **HED028114**
INV : Creatinine

Age : 34 Year | **Sex :** Female
Registered on : 02-09-2026 01:02 PM
Received on : 02-09-2026 06:39 PM
Reported on : 02-09-2026 10:26 PM

SAMPLE COLLECTED AT :
MILLENNIUM PATHOLOGY LAB

SAMPLE : Serum

CREATININE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
CREATININE Method: (Enzymatic)	0.61	mg/dL	0.60 - 1.10

Clinical significance :

Creatinine is a waste product produced by muscles from the breakdown of a compound called creatine. Creatinine is removed from the body by the kidneys, which filter almost all of it from the blood and release it into the urine. This test measures the amount of creatinine in the blood and/or urine.

Creatine is part of the cycle that produces energy needed to contract muscles. Both creatine and creatinine are produced by the body at a relatively constant rate. Since almost all creatinine is filtered from the blood by the kidneys and released into the urine, blood levels are usually a good indicator of how well the kidneys are working. The amount of creatinine you produce depends on your body size and your muscle mass. For this reason, creatinine levels are usually slightly higher in men than in women and children.

The kidneys are a pair of bean-shaped organs that are located at the bottom of the ribcage in the right and left sides of the back. Within them are about a million tiny blood filtering units called nephrons. In each nephron, blood is continually filtered through a microscopic cluster of looping blood vessels, called glomerulus. The glomerulus allows the passage of water and small molecules but retains blood cells and larger molecules. Attached to each glomerulus is a tiny tube (tubule) that collects the fluid and molecules that pass through the glomerulus and then reabsorbs what still can be used by the body. The remaining waste forms urine.

Results from a blood creatinine test may be used in combination with results from other tests, such as a 24-hour urine creatinine test, to calculate values that are used to evaluate kidney function.

Please correlate with clinical conditions

~~End of report~~

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REPORT

PT Name : **MASTER AWESH SHAIKH**
Ref By : **DR. SELF**
Reg No : **HL7801832346 / MH484**
Barcode : **HEN122052**
INV : DENGUE ANTIGEN ELISA NS1

Age : 01 Year | **Sex :** Male
Registered on : 31-08-2026 04:12 PM
Received on : 31-08-2026 06:33 PM
Reported on : 31-08-2026 08:10 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

DENGUE ANTIGEN ELISA NS1

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
DENGUE ELISA NS1	0.19	Index	0.00 - 0.90

Method: ELISA

Clinical significance :

Dengue virus (DV) is a globally distributed flavivirus with 4 distinct serotypes (DV-1, -2, -3, -4) and is primarily transmitted by the *Aedes aegypti* mosquito, found throughout the tropical and subtropical regions of over 100 countries. DV poses a significant worldwide public health threat with approximately 2.5 to 3 billion people residing in DV endemic areas, among whom 100 to 200 million individuals will be infected, and approximately 30,000 patients will succumb to the disease, annually.

Following dengue infection, the incubation period varies from 3 to 7 days, and while some infections remain asymptomatic, the majority of individuals will develop classic dengue fever. Symptomatic patients become acutely febrile and present with severe musculoskeletal pain, headache, retro-orbital pain, and a transient macular rash, most often observed in children. Fever defervescence signals disease resolution in most individuals. However, children and young adults remain at increased risk for progression to dengue hemorrhagic fever and dengue shock syndrome, particularly during repeat infection with a new DV serotype.

Detection of the DV nonstructural protein 1 (NS1) has emerged as an alternative biomarker to both serologic and molecular based techniques for diagnosis of acute DV infection. NS1 antigenemia is detectable within 24 hours and up to 9 days following symptoms onset. This overlaps with the DV viremic phase and NS1 is often detectable prior to IgM seroconversion. Concurrent evaluation for the NS1 antigen alongside testing for IgM- and IgG-class antibodies to DV (DENGM / Dengue Virus Antibody, IgG and IgM, Serum) provides optimal diagnostic potential for both early and late dengue disease.

Interpretation

Positive: The presence of dengue nonstructural protein 1 (NS1) antigen is consistent with acute-phase infection with dengue virus. The NS1 antigen is typically detectable within 1 to 2 days following infection and up to 9 days following symptom onset. NS1 antigen may also be detectable during secondary dengue virus infection, but for a shorter duration of time (1-4 days following symptom onset). Negative: The absence of dengue NS1 antigen is consistent with the lack of acute-phase infection. The NS1 antigen may be negative if specimen is collected immediately following dengue virus infection (<24-48 hours) and is rarely detectable following 9 to 10 days of symptoms. Cautions Results should be used in conjunction with clinical presentation and exposure history. Though uncommon, false-positive nonstructural protein 1 (NS1) results may occur in individuals with active infection due to other flaviviruses, including West Nile virus and yellow fever virus. Negative NS1 antigen results may occur if the specimen was collected greater than 7 days following symptom onset. Serologic testing for the presence of IgM and IgG antibodies to dengue virus is recommended in such cases.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MRS.SULEKHA MONDAL**
Ref By : **DR. NEETI UPASANI**
Reg No : **HL5501832416 / MH466**
Barcode : **HEL067128**
INV : DENGUE ELISA IgG

Age : 24 Year | **Sex :** Female
Registered on : 31-08-2026 04:49 PM
Received on : 31-08-2026 06:42 PM
Reported on : 31-08-2026 07:01 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

DENGUE ELISA IGG

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Dengue IGG Antibodies	0.13	OD Ratio	0.0 - 1.0

Method: ELISA

Clinical significance :

Following dengue infection, the incubation period varies from 3 to 7 days and, while some infections remain asymptomatic, the majority of individuals will develop classic dengue fever. Symptomatic patients become acutely febrile and present with severe musculoskeletal pain, headache, retro-orbital pain, and a transient macular rash, most often observed in children. Fever defervescence signals disease resolution in most individuals. However, children and young adults remain at increased risk for progression to dengue hemorrhagic fever and dengue shock syndrome, particularly during repeat infection with a new DV serotype.

Detection of dengue-specific IgM and IgG-class antibodies remains the most commonly utilized diagnostic method. Seroconversion occurs approximately 3 to 7 days following exposure and, therefore, testing of acute and convalescent sera may be necessary to make the diagnosis. As an adjunct to serologic testing, identification of early DV infection may be made by detection of the DV nonstructural protein 1 (NS1) antigen. NS1 antigenemia is detectable within 24 hours of infection and up to 9 days following symptom onset. The DV NS1 antigen can be detected by ordering DNSAG / Dengue Virus NS1 Antigen, Serum

Interpretation

The presence of IgG-class antibodies to dengue virus (DV) is consistent with exposure to this virus sometime in the past. By 3 weeks following exposure, nearly all immunocompetent individuals should have developed IgG antibodies to DV.

The presence of IgM-class antibodies to DV is consistent with acute-phase infection.

IgM antibodies become detectable 3 to 7 days following infection and may remain detectable for up to 6 months or longer following disease resolution.

The absence of IgM-class antibodies to DV is consistent with lack of infection. However, specimens collected too soon following exposure may be negative for IgM antibodies to DV. If DV remains suspected, a second specimen, collected approximately 10 to 12 days following exposure should be tested.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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Consultant Pathologist (Reg. 0375)

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REPORT

PT Name : **MRS.SULEKHA MONDAL**
Ref By : **DR. NEETI UPASANI**
Reg No : **HL5501832416 / MH466**
Barcode : **HEL067128**
INV : DENGUE ELISA IgM

Age : 24 Year | **Sex :** Female
Registered on : 31-08-2026 04:49 PM
Received on : 31-08-2026 06:42 PM
Reported on : 31-08-2026 07:00 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

DENGUE ELISA IGM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Dengue IGM Antibodies Method: ELISA	0.16	OD Ratio	0.0 - 1.0

Clinical significance :

Following dengue infection, the incubation period varies from 3 to 7 days and, while some infections remain asymptomatic, the majority of individuals will develop classic dengue fever. Symptomatic patients become acutely febrile and present with severe musculoskeletal pain, headache, retro-orbital pain, and a transient macular rash, most often observed in children. Fever defervescence signals disease resolution in most individuals. However, children and young adults remain at increased risk for progression to dengue hemorrhagic fever and dengue shock syndrome, particularly during repeat infection with a new DV serotype.

Detection of dengue-specific IgM and IgG-class antibodies remains the most commonly utilized diagnostic method. Seroconversion occurs approximately 3 to 7 days following exposure and, therefore, testing of acute and convalescent sera may be necessary to make the diagnosis. As an adjunct to serologic testing, identification of early DV infection may be made by detection of the DV nonstructural protein 1 (NS1) antigen. NS1 antigenemia is detectable within 24 hours of infection and up to 9 days following symptom onset. The DV NS1 antigen can be detected by ordering DNSAG / Dengue Virus NS1 Antigen, Serum.

Interpretation

The presence of IgG-class antibodies to dengue virus (DV) is consistent with exposure to this virus sometime in the past. By 3 weeks following exposure, nearly all immunocompetent individuals should have developed IgG antibodies to DV.

The presence of IgM-class antibodies to DV is consistent with acute-phase infection.

IgM antibodies become detectable 3 to 7 days following infection and may remain detectable for up to 6 months or longer following disease resolution.

The absence of IgM-class antibodies to DV is consistent with lack of infection. However, specimens collected too soon following exposure may be negative for IgM antibodies to DV. If DV remains suspected, a second specimen, collected approximately 10 to 12 days following exposure should be tested.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name : **MS FARIDA PARVIN**
Ref By : **DR. SELF**
Reg No : **HL5401827392 / MH037**
Barcode : **HEN163504**
INV : HY Coagulation Profile -1

Age : 21 Year | **Sex :** Male
Registered on : 26-08-2026 03:18 PM
Received on : 26-08-2026 05:16 PM
Reported on : 27-08-2026 03:24 PM

SAMPLE COLLECTED AT :
SELF

SAMPLE : Citrated Plasma

FIBRINOGEN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Fibrinogen Antigen Method: Immunoturbidimetric	314.5	mg/dL	196.00 - 441.00

Clinical significance :
Fibrinogen (clotting factor I) is an essential protein responsible for blood clot formation. In the final step of the coagulation cascade, thrombin converts soluble fibrinogen into insoluble fibrin strands that crosslink and form a clot.

Fibrinogen is synthesized in the liver and has a biological half-life of 3 to 5 days in the circulating plasma. Fibrinogen deficiencies can be congenital or acquired and lead to prolonged coagulation times. Isolated fibrinogen deficiency is an extremely rare inherited coagulation disorder.

Acquired fibrinogen deficiency is most commonly caused by, acute or decompensated intravascular coagulation and fibrinolysis (DIC). Other causes of fibrinogen deficiency include advanced liver disease, L-asparaginase therapy, or fibrinolytic agents (eg, streptokinase, urokinase, tissue plasminogen activator

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **JOSANA URADE**	**Age :** 50 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** SMBS HOSPITAL
Ref By	: **DR. SMBS HOSPITAL , ZADPOLI**	**Registered on :** 25-08-2026 10:09 PM	
Reg No	: **HL6701826519 / MH004/012/SAMBRE2025**	**Received on** : 27-08-2026 06:39 PM	
Barcode	: **HEE176917**	**Reported on** : 27-08-2026 06:40 PM	
INV	: Fluid Cytology		**SAMPLE :** Body Fluid

FLUID CYTOLOGY

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

INTERNATIONAL SYSTEM FOR REPORTING FLUID CYTOPATHOLOGY-

MICROSCOPIC PICTURE -SMEARS STUDIED SHOWS BENIGN AND REACTIVE COMPONENTS: MESOTHELIAL AND INFLAMMATORY CELLS
REACTIVE MESOTHELIAL CELLS ARE TYPICALLY SINGLE CELLS OR SMALL CLUSTERS AND EXHIBIT BINUCLEATION, MULTINUCLEATION AND REACTIVE CELLULAR ATYPIA; MESOTHELIAL CELLS PRESENT AS SHEETS , NO SECOND LINE CELLS PRESENT.

INTERPRETATIONS- SUGGESTIVE OF INFLAMMATORY SMEAR .

COMMENT –NO MALIGNANT CELLS SEEN IN THE STUDIED SLIDE .

~~End of report~~

Dr. NAMARATA SHARAN (MD PATH)
Consultant Pathologist (Reg. 41002)

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REPORT

PT Name	: **SANGEETA KOILKAR**	**Age :** 48 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** SHRIKANT MURCHAVDE
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 09:38 AM	
Reg No	: **HL3801829851 / MH031**	**Received on** : 29-08-2026 04:30 PM	
Barcode	: **HEN199498**	**Reported on** : 31-08-2026 11:03 PM	**SAMPLE :** Body Fluid
INV	: Gene Xpert (CB NAAT) FLUID		

GENE XPERT (CB NAAT) FLUID

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Mycobacterium Tuberculosis Method: Semi - Nested Real Time PCR	NOT DETECTED	.	. - .

Clinical significance :

Result Interpretation:

- **If result is TB negative: This indicates no tuber culosis (TB). However, if the clinical picture is strongly** suggestive of TB, a repeat test can be suggested by your referring doctor.

If result is TB positive, and Rifampicin resistan ce negative: This indicates tuberculosis that can be treated with commonly used drugs. Please seek assistance from the referring doctor for treatment of tuberculosis, which is a highly curable disease.

- If result is Rifampicin resistance positive and you have previously taken TB treatment: This indicates drug resistance - TB that is difficult to treat with commonly used drugs. Please seek assistance from the referring doctor for treatment of multidrug resistant tuberculosis (MDR TB) which requires specialized management,including additional drug-susceptibility testing.

- If result is Rifampicin resistance positive and you have NOT previously taken TB treatment: This may indicate drug resistance, but requires confirmation. Please seek a second test from an accredited laboratory to confirm the presence of multidrug resistant tuberculosis (MDR TB) which requires specialized management.

You are entitled to FREE TB drugs from specific government TB centers and hospitals.Details are available on www.ipaqt.org.

Clinical Background:
GeneXpert MTB/RIF, is a rapid automated molecular test for diagnosis of Mycobacterium tuberculosis (MTB) and resistance to rifampicin (RIF) in clinical specimens.
It targets rpoB gene (hot spot region) which is critical for identifying Rifampicin resistance mutations.

- MDR TB is defined as TB due M.tb complex,resistant to both Isoniazid & Rifampicin. Rifiampicin

acts as a surrogate marker for MDR TB as 90% of rifampicin resistant strains are also isoniazid resistant.

~~End of report~~

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REPORT

PT Name	: **MR. HABIBULLAH CHAUDHARY**	**Age :** 65 Year | **Sex :** Male		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. LIFE CARE PATHOLOGY**	**Registered on**	: 29-08-2026 08:49 PM	LIFE CARE PATHOLOGY	
Reg No	: **HL2101830634 / MH713**	**Received on**	: 29-08-2026 09:00 PM		
Barcode	: **HEN143911**	**Reported on**	: 31-08-2026 03:17 PM		
INV	: Gene Xpert (CB NAAT) PUS			**SAMPLE :** PUS	

GENE XPERT (CB NAAT) PUS

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Mycobacterium Tuberculosis Method: Semi - Nested Real Time PCR	NOT DETECTED	.	. - .

Clinical significance :
Result Interpretation:

- **If result is TB negative: This indicates no tuber culosis (TB). However, if the clinical picture is strongly** suggestive of TB, a repeat test can be suggested by your referring doctor.

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- If result is Rifampicin resistance positive and you have previously taken TB treatment: This indicates drug resistance - TB that is difficult to treat with commonly used drugs. Please seek assistance from the referring doctor for treatment of multidrug resistant tuberculosis (MDR TB) which requires specialized management,including additional drug-susceptibility testing.

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- MDR TB is defined as TB due M.tb complex,resistant to both Isoniazid & Rifampicin. Rifiampicin

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Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **ROZINA**	**Age :** 24 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** LIFECARE CLINICAL LAB - 9325766190
Ref By	: **DR. .....**	**Registered on** : 29-08-2026 04:33 PM	
Reg No	: **HL8001830436 / RAJ06**	**Received on** : 29-08-2026 06:13 PM	
Barcode	: **HEA182435**	**Reported on** : 29-08-2026 08:15 PM	**SAMPLE :** Serum
INV	: Indirect Coombs Test (ICT)		

INDIRECT COOMBS TEST (ICT)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
COOMBS TEST, INDIRECT Method: Erythrocyte Magnetized Technology	NEGATIVE	.	. - .

Clinical significance :

If a pregnant woman with Rh-negative blood is pregnant with a baby (fetus) with Rh-positive blood, Rh sensitization may occur. The baby may have Rh-positive blood if the father has Rh-positive blood. Rh sensitization happens when the baby's blood mixes with the mother's blood during pregnancy or delivery. This causes the mother's immune system to make antibodies against the baby's red blood cells in future pregnancies. This antibody response is called Rh sensitization and, depending on when it happens, can destroy the red blood cells of the baby before or after it is born. If sensitization happens, a fetus or newborn can develop mild to severe problems (called Rh disease or erythroblastosis fetalis). In rare cases, if Rh disease is not treated, the fetus or newborn may die.

A woman with Rh-negative blood can get a shot of Rh immunoglobulin (such as RhoGAM) that almost always stops sensitization from occurring. Problems from Rh sensitization have become very rare since Rh immunoglobulin was developed.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
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REPORT

PT Name	: **SACHIN SAWANT**	**Age :** 39 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. ANGEL PATHOLOGY LAB**	**Registered on** : 29-08-2026 01:53 PM	ANGEL PATHOLOGY LAB BEED 9860018831
Reg No	: **HL7901830185 / MH316**	**Received on** : 30-08-2026 09:06 AM	
Barcode	: **HEB188540**	**Reported on** : 30-08-2026 01:10 PM	
INV	: inorganic Phosphorus		**SAMPLE :** Serum

INORGANIC PHOSPHORUS

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
inorganic phosphorus Method: (PHOTOMETRY)	7.02	mg/dL	. - .

Reference Range

Males
1-4 years: 4.3-5.4 mg/dL
5-13 years: 3.7-5.4 mg/dL
14-15 years: 3.5-5.3 mg/dL
16-17 years: 3.1-4.7 mg/dL
> or =18 years: 2.5-4.5 mg/dL
Reference values have not been established for patients that are less than 12 months of age.
Females
1-7 years: 4.3-5.4 mg/dL
8-13 years: 4.0-5.2 mg/dL
14-15 years: 3.5-4.9 mg/dL
16-17 years: 3.1-4.7 mg/dL
> or =18 years: 2.5-4.5 mg/dL
Reference values have not been established for patients that are less than 12 months of age.

Clinical significance :

Of the phosphorus contained in the body, 88% is localized in bone in the form of hydroxyapatite. The remainder is involved in intermediary carbohydrate metabolism and in physiologically important substances such as phospholipids, nucleic acids, and adenosine triphosphate (ATP). Phosphorus occurs in blood in the form of inorganic phosphate and organically bound phosphoric acid. The small amount of extracellular organic phosphorus is found exclusively in the form of phospholipids. Serum contains approximately 2.5 to 4.5 mg/dL of inorganic phosphate (the fraction measure in routine biochemical assays). Serum phosphate concentrations are dependent on meals and variation in the secretion of hormones such as parathyroid hormone (PTH) and may vary widely.

Hypophosphatemia may have 4 general causes: shift of phosphate from extracellular to intracellular, renal phosphate wasting, loss from the gastrointestinal tract, and loss from intracellular stores.

Hyperphosphatemia is usually secondary to an inability of the kidneys to excrete phosphate. Other factors may relate to increased intake or a shift of phosphate from the tissues into the extracellular fluid.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **SHARAD GOLE**	**Age :** 45 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. PETKAR HOSPITAL**	**Registered on :** 28-08-2026 07:11 AM	
Reg No	: **HL4801828822 / PUN104**	**Received on** : 28-08-2026 08:00 AM	
Barcode	: **Hen183499**	**Reported on** : 30-08-2026 01:09 PM	**SAMPLE :** PUS
INV	: PUS MICROSCOPY		

PUS MICROSCOPY

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE

SPECIMEN: PUS MICROSCOPY

MICROSCOPY: Smears show abundant neutrophils, necrotic debris, and proteinaceous background. No atypical or malignant cells are identified.

Impression: Acute suppurative inflammatory lesion (pus). Correlation with Gram stain, culture and sensitivity is recommended.

~~End of report~~

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REPORT

PT Name	: **ALFIYA SHAIKH**	**Age :** 27 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 24-08-2026 12:33 PM	
Reg No	: **HL4601824375 / RAJ35**	**Received on** : 24-08-2026 10:12 PM	
Barcode	: **HEN121303**	**Reported on** : 25-08-2026 05:22 PM	**SAMPLE :** Serum
INV	: SERUM CERULOPLASMIN		

SERUM CERULOPLASMIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Ceruloplasmin Method: Nephelometric Assay	32.4	mg/dL	. - .

Reference Range

Males:
0-8 weeks: 7.4-23.7 mg/dL
9 weeks-5 months: 13.5-32.9 mg/dL
6-11 months: 13.7-38.9 mg/dL
12 months-7 years: 21.7-43.3 mg/dL
8-13 years: 20.5-40.2 mg/dL
14-17 years: 17.0-34.8 mg/dL
> or =18 years: 19.0-31.0 mg/dL

Females:
0-8 weeks: 7.4-23.7 mg/dL
9 weeks-5 months: 13.5-32.9 mg/dL
6-11 months: 13.7-38.9 mg/dL
12 months-7 years: 21.7-43.3 mg/dL
8-13 years: 20.5-40.2 mg/dL
14-17 years: 20.8-43.2 mg/dL
> or = 18 years: 20.0-51.0 mg/dl

Clinical significance :

Ceruloplasmin is a positive acute-phase reactant and a copper-binding protein that accounts for over 95% of serum copper in normal adults. Ceruloplasmin is measured primarily to assist with a diagnosis of Wilson disease. Other indications include Menkes disease, dietary copper insufficiency, and risk of cardiovascular disease.

Wilson disease is a rare inherited disorder of copper transport that results in low serum copper and ceruloplasmin and accumulation of copper in various tissues. The pathological accumulation of copper in the liver, brain, cornea, and kidney causes cirrhosis, neuropsychiatric symptoms, Kayser-Fleischer rings, and hematuria/proteinuria, respectively. See Wilson Disease Testing Algorithm in Special Instructions for appropriate use of clinical findings, serum biomarkers, genetic tests, and tissue biopsies when working up suspected cases.

Menkes disease is an X-linked disorder in which dietary copper is absorbed from the gastrointestinal tract but cannot be transported, so copper is not available to the liver for incorporation into ceruloplasmin.

Dietary ceruloplasmin deficiency may be due to inadequate dietary copper intake, long-term parenteral nutrition without copper supplementation, malabsorption, penicillamine therapy, or a combination of these

Please correlate with clinical conditions

~~End of report~~

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REPORT

PT Name	: **ANIL ROHRA**	**Age :** 46 Year | **Sex :** Male	**SAMPLE COLLECTED AT :** BLUECLIFFE LABORATORY
Ref By	: **DR. SELF**	**Registered on :** 30-08-2026 02:40 PM	
Reg No	: **HL3601831288 / RAJ04**	**Received on** : 30-08-2026 03:25 PM	
Barcode	: **HEE156270**	**Reported on** : 30-08-2026 08:13 PM	**SAMPLE :** STOOL
INV	: stool For Occult Blood		

STOOL FOR OCCULT BLOOD

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Stool For Occult Blood	NEGATIVE	.	. - .

Method: .

Clinical significance :

Blood in the stool may also be a sign of colorectal cancer, a type of cancer that starts in the colon or rectum. Colorectal cancer is the second leading cause of cancer-related deaths in the United States and the third most common cancer in men and in women. A fecal occult blood test is a screening test that may help find colorectal cancer early, when treatment is most effective

Please correlate with clinical conditions

~~End of report~~

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REPORT

PT Name : **MRS. JUGRAFIYA**
Ref By : **DR. AMEER PATIL**
Reg No : **HL2501827023 / MH552**
Barcode : **HEN110284**
INV : TROPONIN-T

Age : 40 Year | **Sex :** Female
Registered on : 26-08-2026 12:26 PM
Received on : 26-08-2026 04:39 PM
Reported on : 26-08-2026 06:10 PM

SAMPLE COLLECTED AT :
AMISHA LAB

SAMPLE : EDTA Blood

TROPONIN-T

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TROPONIN-T	NEGATIVE	.	. - .

Method: .

Clinical significance :

Troponin T is found in free cytosol and structurally bound protein. The unbound pool of troponin T is the source of early protein release in myocardial damage. Troponin T is released from the structural elements at a later stage, corresponding to the degradation of myofibrils that occurs in irreversible myocardial damage. Troponin T becomes elevated 2 to 4 hours after the onset of myocardial necrosis and can remain elevated for up to 14 days, or even longer on occasion.

The most common cause of cardiac injury is myocardial ischemia, ie, acute myocardial infarction. These patients are known to have an adverse short- and long-term prognosis compared to patients with unstable angina and no elevation of troponin T. Many of these patients, especially those with troponin T elevations above 30 ng/L, benefit from an aggressive strategy with anticoagulation and an invasive interventional strategy.

Please correlate with clinical conditions

~~End of report~~

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REPORT

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SAMPLE COLLECTED AT :
AMISHA LAB

SAMPLE : EDTA Blood

TROPONIN-T

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TROPONIN-T	NEGATIVE	.	. - .

Method: .

Clinical significance :

Troponin T is found in free cytosol and structurally bound protein. The unbound pool of troponin T is the source of early protein release in myocardial damage. Troponin T is released from the structural elements at a later stage, corresponding to the degradation of myofibrils that occurs in irreversible myocardial damage. Troponin T becomes elevated 2 to 4 hours after the onset of myocardial necrosis and can remain elevated for up to 14 days, or even longer on occasion.

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Please correlate with clinical conditions

~~End of report~~

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